Chronic Care Management (CCM) Workflow and Operations Guide

Standard Operating Procedures for Clinical, Administrative and Billing Teams

Version 1.0 | Owner: Program Medical Director | Review cycle: Every 12 months

1. Purpose, Scope and How to Use This Guide

1.1 Purpose

This guide defines exactly how the practice runs its Chronic Care Management (CCM) program from the first day a patient is identified to the day they leave the program. It is written so that a new team member can read a section and perform the task correctly without guessing. Every procedure has an owner, a trigger, a set of steps, a time standard and a documentation requirement.

1.2 Scope

This guide covers:

- Identifying and screening eligible patients
- Explaining the program and obtaining consent
- Completing the initial assessment and building the comprehensive care plan
- The monthly care management cycle (outreach, monitoring, medication review, coordination)
- Escalation of clinical concerns and emergencies
- Care transitions (hospital, emergency department, specialist, skilled nursing)
- Time tracking, documentation and billing
- Quality measurement, audits, privacy and disenrollment

This guide does not replace clinical judgment. Where a patient's safety is at stake, clinical judgment and the escalation protocol in Section 12 always take priority over workflow convenience.

1.3 Assumptions

- The practice is an outpatient primary care or specialty practice with a certified electronic health record (EHR).
- The program follows US Medicare CCM rules. Billing details in Section 14 are specific to Medicare Part B; practices outside the US should keep the clinical workflow and replace the billing section with their local payer rules.
- Payer rules, codes and rates change. The billing lead must verify current requirements each January and whenever the payer publishes updates.

1.4 How to Use This Guide

If you are a...	Read first	Then
Care coordinator or nurse	Sections 4 to 12	Section 13
Physician or advanced practice provider	Sections 3, 7, 12	Sections 8, 14
Front desk or scheduler	Sections 4, 5	Section 9
Billing and compliance staff	Sections 13 to 16	Appendix A
Program manager	All sections	Appendices

2. Program Overview and Definitions

2.1 What Chronic Care Management Is

CCM is a non-face-to-face care service delivered between office visits to patients who live with multiple serious chronic conditions. A designated care team provides continuous oversight, a written comprehensive care plan, medication management, coordination with other providers, and 24/7 access to urgent help.

2.2 Program Goals

1. Prevent avoidable emergency visits and hospital admissions.
2. Improve control of chronic conditions (for example blood pressure, blood glucose, symptom burden).
3. Improve medication adherence and reduce medication errors.
4. Give every enrolled patient one known point of contact.

5. Keep every patient's care plan current and shared with everyone who treats them.

2.3 Key Definitions

Term	Meaning in this guide
Chronic condition	A condition expected to last at least 12 months or until the patient's death
Eligible patient	Has two or more chronic conditions that place them at significant risk of death, acute exacerbation or decompensation, or functional decline
Billing practitioner	The physician or qualified provider who bills for CCM and holds clinical responsibility
Clinical staff	Nurses, medical assistants and other staff working under general supervision of the billing practitioner within their licensed scope
Care coordinator	The named team member who is the patient's main point of contact
Comprehensive care plan	The written, patient-centered plan covering problems, goals, interventions, medications, resources and responsible parties
Initiating visit	A qualifying face-to-face visit required before CCM starts for new patients or patients not seen within the prior year
Calendar month	The billing and time-tracking period (first day to last day of the month)
Escalation	Passing a concern to a higher level of clinical review within a set time

2.4 Program Lifecycle at a Glance

1. Identify: find eligible patients from EHR registries.
2. Verify: confirm diagnoses, insurance and visit requirements.
3. Enroll: explain the program, obtain and document consent.

4. Assess: complete the comprehensive initial assessment.
5. Plan: build, sign and share the care plan.
6. Manage: run the monthly cycle.
7. Coordinate: manage transitions and specialist communication.
8. Document and bill: record time and submit compliant claims.
9. Review: audit quality and update the plan.
10. Exit: disenroll when the patient requests, becomes ineligible or leaves the practice.

3. Roles and Responsibilities

3.1 Role Summary

Role	Primary responsibilities	Authority
Program Medical Director	Clinical oversight, protocol approval, resolves complex clinical questions	Approves all clinical protocols
Billing Practitioner	Establishes and signs care plan, supervises staff, personally reviews high-risk patients	Orders, prescriptions, diagnosis changes
Care Coordinator (RN, LPN or trained MA)	Monthly outreach, symptom review, plan updates, coordination, time logging	Works within licensed scope and standing protocols
Care Team Assistant	Scheduling, records requests, reminders, resource referrals	Non-clinical tasks only
Front Desk	Identifies candidates at check-in, first explanation of program, appointment booking	Non-clinical tasks only
Billing and Compliance Lead	Eligibility checks, claims, audits, rule updates	Holds the right to hold any claim that fails the checklist
Program Manager	Staffing, caseload balancing, reporting, training	Assigns caseloads

Role	Primary responsibilities	Authority
IT and Privacy Officer	Access control, secure messaging, audit logs	Controls system permissions

3.2 Caseload Standards

- Target caseload per full-time care coordinator: 150 to 250 enrolled patients depending on patient complexity and support staff. Review quarterly.
- No coordinator should carry more than 20 percent high-risk patients without the program manager adjusting the total caseload downward.
- Every patient must have one named primary coordinator and one named backup.

3.3 Supervision Rules

1. Clinical staff work under the general supervision of the billing practitioner.
2. Any task outside a coordinator's licensed scope must be routed to a provider.
3. The billing practitioner reviews and signs each new care plan and each major revision.
4. High-risk patients receive a documented provider review at least monthly.

3.4 Daily Handoffs

At the start of each day the coordinator team holds a 10-minute huddle covering: new hospital discharges, patients flagged red on yesterday's outreach, patients who could not be reached, appointments requiring preparation, and staffing coverage.

4. Patient Identification and Eligibility

4.1 Sources for Identifying Candidates

- EHR registry report run on the first business day of each month
- Provider referral during or after a visit
- Hospital discharge and emergency department notifications
- Front-desk and nurse observations at check-in
- Patient or family request

4.2 Clinical Eligibility Screen

A patient is a candidate when all of the following are true:

1. They have two or more chronic conditions expected to last at least 12 months or until death.
2. The conditions place the patient at significant risk of death, acute exacerbation or decompensation, or functional decline.
3. A provider confirms the diagnoses in the record.

Common qualifying conditions include hypertension, type 2 diabetes, heart failure, COPD, asthma, chronic kidney disease, coronary artery disease, depression, dementia, osteoporosis, arthritis and chronic pain conditions. The provider decides eligibility; the list is a guide, not a limit.

4.3 Administrative Eligibility Screen

Before any outreach, the billing lead confirms:

Check	Pass condition	If it fails
Active coverage	Patient has active coverage that supports CCM	Do not enroll; note reason
Initiating visit	Seen by the billing practitioner within the last 12 months, or a visit is scheduled	Book a qualifying visit first
No conflicting service	Patient is not enrolled in a service the payer bars from being billed in the same month (for example hospice or certain home health or facility-based arrangements)	Hold; ask billing lead to review the current payer list
No duplicate CCM	No other practitioner billed CCM for the patient this month	Contact patient and the other practice; one billing practitioner per month only
Cost sharing understood	Patient's expected copay or coinsurance identified	Include in the consent conversation

4.4 Candidate Prioritization

If demand exceeds capacity, enroll in this order:

1. Recent hospital or emergency department use in the last 6 months
2. Five or more chronic conditions or ten or more medications
3. Poorly controlled markers (for example HbA1c above target, uncontrolled blood pressure)
4. Documented adherence or social barriers
5. All other eligible patients

4.5 Output of This Step

A verified candidate list with the provider's eligibility sign-off, assigned coordinator, and the date the enrollment call or visit conversation will occur.

5. Enrollment and Consent Workflow

5.1 Who Conducts the Conversation

The coordinator or the billing practitioner explains the program. The front desk may introduce it but must not obtain consent alone.

5.2 Required Information to Tell the Patient

Before consent is accepted, the patient must be told all of the following. This is a regulatory requirement, so no item is optional:

1. The service exists and what it includes (monthly care management, care plan, 24/7 access, coordination).
2. They may owe cost sharing (copay or coinsurance) and the likely amount.
3. Only one practitioner can bill for CCM in a calendar month.
4. They may stop CCM at any time, effective at the end of the calendar month.
5. Their health information will be shared with other treating providers as needed for coordination.

5.3 Suggested Script

'We have a program for patients with several ongoing health conditions. A nurse from our team will be your main contact. Each month we call to check how you are doing, review your medicines, help with appointments and make sure all your doctors are on the same page. You will also have a number to call any time, day or night, for urgent concerns. Your insurance may charge a small monthly amount of about [amount]. Only one practice can provide this service for you at a time, and you can stop whenever you like; it would end at the close of that month. May I enroll you?'

5.4 Obtaining and Documenting Consent

1. Consent may be verbal or written.
2. Record in the EHR: date, name of the person who obtained consent, method (verbal or written), confirmation that all five items in 5.2 were explained, and the patient's decision.
3. If written, scan the signed form into the chart.
4. If the patient has a legal representative, obtain consent from them and record their relationship and authority.
5. If the patient declines, document it. Do not re-approach for 90 days unless the patient asks or clinical status changes.

5.5 Enrollment Completion Checklist

- [ ] Provider eligibility sign-off present
- [ ] Initiating visit completed or scheduled
- [ ] Consent explained and documented
- [ ] Primary and backup coordinator assigned
- [ ] Patient given the 24/7 access number and the coordinator's direct line
- [ ] Preferred contact method, language and best time recorded
- [ ] Caregiver or authorized contact recorded with permission
- [ ] Enrollment date entered in the registry
- [ ] Initial assessment appointment set within 14 days

5.6 Welcome Packet

Within 5 business days of enrollment, send the patient (in their preferred language and format) a welcome packet containing: program summary, coordinator name and phone number, after-hours number, what to do in an emergency, how to reach the team by secure message, and a copy of the cost-sharing and cancellation terms.

6. Initial Comprehensive Assessment

6.1 Purpose and Timing

The initial assessment collects everything needed to build the care plan. Complete it within 14 days of enrollment. It can be done by phone, video or in person; the coordinator leads it and the provider reviews the results.

6.2 Assessment Domains

Domain	What to collect
Medical history	Active chronic conditions, recent hospital or emergency use, surgeries, allergies
Symptoms	Current symptom burden for each condition, red-flag symptoms
Medications	Full reconciled list including over-the-counter, supplements and herbal products
Functional status	Ability to perform daily activities (bathing, dressing, eating, mobility, medicine handling)
Cognition and mood	Brief screens for depression and memory concerns as clinically appropriate
Home and safety	Falls risk, living situation, who lives with the patient, home safety concerns
Social needs	Food, transport, housing, finances, utilities, health literacy, language
Caregivers	Who helps, their contact details, their own capacity and stress

Domain	What to collect
Preventive care	Vaccinations, screenings, eye and foot exams, dental
Patient goals	What matters most to the patient, in their own words
Advance care planning	Whether the patient has a health care proxy, living will or code status documented
Care team	All treating providers, pharmacies and specialists with contact details

6.3 Step-by-Step Procedure

1. Review the chart before the call: problem list, last three visits, recent labs, current medication list, hospital summaries.
2. Call or meet the patient at the agreed time. Confirm identity using two identifiers.
3. Work through each domain in 6.2, asking open questions first.
4. Ask the patient to read medication bottle labels aloud (or have a caregiver do so) for the reconciliation.
5. Ask: 'What are the one or two things you most want help with?' Record the answer verbatim.
6. Score the risk level using the tool in 6.4.
7. Record all findings in the EHR assessment template the same day.
8. Send the completed assessment to the billing practitioner for review within 2 business days.

6.4 Risk Stratification

Assign each patient a tier at enrollment and re-score every quarter or after any hospitalization.

Tier	Typical features	Minimum contact
Low (green)	Conditions stable, few medications, good support, no recent acute use	Monthly call

Tier	Typical features	Minimum contact
Moderate (yellow)	One or more conditions unstable, 5 to 9 medications, some social or adherence barriers	Every 2 weeks
High (red)	Hospital or emergency use in the last 90 days, multiple unstable conditions, 10 or more medications, cognitive impairment, or limited support	Weekly until stable, then every 2 weeks

The provider may raise any patient's tier at any time. Lowering a patient from red requires provider approval.

6.5 Output of This Step

A completed, dated assessment in the chart with a risk tier, a reconciled medication list, a prioritized problem list and a short note of the patient's goals, ready to become the care plan.

7. The Comprehensive Care Plan

7.1 Purpose

The care plan is the single source of truth for what the team and the patient are working toward. It is a living document, patient-centered, written in plain language, and available to the patient and every treating provider.

7.2 Required Contents

Element	Description
Problem list	Every chronic condition and its current status
Expected outcome and prognosis	What the provider expects and what is realistic for this patient
Measurable treatment goals	Specific targets, for example blood pressure below an agreed level, HbA1c at the provider's target, no emergency visits in 6 months

Element	Description
Patient goals	The patient's own priorities in their words
Symptom management	What to do for each condition when symptoms change
Planned interventions	Who does what, and by when, including provider and community resources
Medication management	Reconciled list, purpose of each drug, adherence supports
Community and social services	Transport, food support, home help, caregiver support and their status
Coordination plan	Specialists, pharmacies, home health or other services and who communicates with them
Preventive care schedule	Vaccines, screenings and exams with due dates
Advance care planning	Proxy, living will and code status documentation
Review schedule	Date of next planned review

7.3 Building the Plan: Procedure

1. The coordinator drafts the plan from the assessment within 5 business days of completing it.
2. The coordinator lists the top three priorities by combining clinical urgency with the patient's stated goals.
3. For each priority, write one measurable goal, one or two interventions, an owner and a target date.
4. The billing practitioner reviews, edits and signs the plan. Date and signature must appear in the record.
5. The patient reviews the plan with the coordinator (by phone, portal or mail) and confirms it reflects their wishes.
6. A copy is given to the patient or caregiver in their preferred format. This is a required element; record the date and method of delivery.
7. Share the plan electronically with other treating providers as appropriate for coordination, and record that it was sent.

7.4 Example Goal Statements

Weak goal	Strong goal
Control diabetes	Patient checks blood sugar fasting 4 days per week and logs it; HbA1c reaches the provider's target by the next quarterly lab
Take medicines	Patient uses a weekly pill organizer filled every Sunday; zero missed doses reported at monthly check-ins for 3 months
Avoid hospital	Patient knows the three warning signs of heart failure worsening and calls the team within 24 hours of any sign

7.5 Updating the Plan

The care plan is reviewed:

- At every monthly cycle (brief check that nothing has changed)
- Within 7 days after any hospital, emergency or skilled nursing stay
- After any medication change that affects goals
- At any new diagnosis or significant change in function
- At least once every 12 months as a full rewrite with provider re-signature

Every version is dated and retained. The patient receives a fresh copy whenever the content changes in a meaningful way.

8. The Monthly Operating Cycle

This section is the heart of the program. Every enrolled patient passes through this cycle every calendar month. The cycle is built around a fixed calendar so that nothing is missed and billing thresholds are met without last-minute rushing.

8.1 Monthly Calendar

Business day	Task	Owner
Day 1	Run registry and enrollment reports; confirm active list; update tiers; flag disenrollments	Program manager
Days 1 to 3	Review each patient's chart for new labs, visits, discharge notices and pending referrals; build the month's outreach list	Care coordinator
Days 2 to 10	First outreach to every patient (weekly for red, biweekly for yellow)	Care coordinator
Days 5 to 15	Medication review and reconciliation for patients with changes; pharmacy contact as needed	Coordinator with provider
Days 10 to 20	Coordination tasks: specialist communication, referrals, social services, equipment	Care team assistant and coordinator
Day 15	Mid-month check: any patient with fewer than 10 minutes logged is prioritized	Program manager
Days 20 to 25	Follow-up outreach; care plan updates; provider review of red-tier patients	Coordinator and provider
Last 3 business days	Time log audit; close open tasks; list patients who missed the time threshold	Billing lead
First 3 days of next month	Submit claims for patients who met all requirements	Billing lead

8.2 Standard Monthly Patient Encounter (Step by Step)

1. Prepare (3 to 5 minutes). Open the chart. Review the care plan, last month's note, new results, upcoming appointments, hospital notifications and any open tasks.

2. Start the timer in the EHR time-tracking tool and note the patient's name and the activity type.
3. Contact the patient. Confirm identity with two identifiers. Use the call standards in Section 9.
4. Ask about overall status: 'How have you been since we last spoke?'
5. Review each chronic condition using its symptom checklist (see 8.3). Record answers in the structured template.
6. Check home readings if applicable (blood pressure, glucose, weight, oxygen). Compare them against the targets in the care plan.
7. Review medicines: any missed doses, side effects, refills due, new prescriptions from other providers, new over-the-counter products.
8. Ask about appointments, transport and upcoming tests. Resolve barriers now if possible.
9. Ask about social and home needs: food, safety, caregiver strain, mood, loneliness.
10. Reinforce one or two self-management tips relevant to the patient's goals. Use teach-back: ask the patient to repeat the plan in their own words.
11. Agree on the next contact date and what the patient will do before then.
12. End the call and stop the timer. Record the total minutes.
13. Write the encounter note the same day (see 13.3).
14. Create tasks for follow-up items with owners and due dates. Escalate anything meeting the criteria in Section 12.

8.3 Condition-Specific Check Items

Condition	Ask about	Escalate if
Hypertension	Home readings, headache, dizziness, missed doses, salt intake	Reading above the provider-set urgent threshold, severe headache or chest pain
Type 2 diabetes	Fasting and post-meal readings, low-sugar episodes, foot problems, eye symptoms, diet	Repeated readings outside provider limits, any low-sugar event needing help, foot wound

Condition	Ask about	Escalate if
Heart failure	Daily weight, swelling, breathlessness, pillows needed at night, salt and fluid intake	Weight gain beyond the provider's limit in a day or a week, worsening breathlessness
COPD or asthma	Cough, sputum change, rescue inhaler use, activity tolerance, triggers	Increased rescue use, breathlessness at rest, fever with sputum change
Chronic kidney disease	Swelling, urine output, avoided medicines, diet, lab dates	New swelling, sharply reduced urination, new nausea or confusion
Depression or anxiety	Mood, sleep, interest, safety	Any thought of self-harm: follow Section 12.4 immediately
Chronic pain	Pain score, function, sleep, side effects, adherence to plan	Uncontrolled pain, new severe symptoms, signs of misuse or overuse
Dementia or cognitive decline	Memory, safety, wandering, caregiver stress	Safety incident, caregiver crisis, sudden change in mental state

Thresholds for escalation come from the patient's own care plan and the provider's orders, not from this table alone.

8.4 Outreach Attempts and Unreachable Patients

1. Make the first attempt at the patient's preferred time and method.
2. If there is no answer, leave a minimum-necessary voicemail without health details: name, practice, call-back number.
3. Make a second attempt within 2 business days at a different time of day.
4. Make a third attempt by an alternate method (text if consented, secure message, caregiver contact).
5. After three failed attempts across 7 days, send a letter and notify the provider if the patient is yellow or red.
6. If a red-tier patient cannot be reached for 7 days, the coordinator requests a welfare check approach approved by the provider (for example contacting the emergency contact or a home visit request).

7. Record every attempt with date, time and method. Time spent on attempts counts only when it is clinical staff work directed toward the care plan; the billing lead decides what is countable under current rules.

8.5 Monthly Closeout Standard

Before the month ends, each patient record must show:

- [ ] At least one documented contact or a documented attempt sequence
- [ ] Updated medication list status
- [ ] Care plan reviewed and changes dated
- [ ] All open tasks assigned or closed
- [ ] Total minutes logged and correct
- [ ] Provider review documented for red-tier patients
- [ ] Next contact date scheduled

9. Patient Communication Standards

9.1 Principles

- Use plain language. Avoid jargon. Aim for a reading level suitable for the patient.
- Use the patient's preferred language. Use a qualified interpreter when the coordinator is not fluent; do not use family members as interpreters for clinical content unless the patient insists.
- Speak with respect and without hurry. Allow silence.
- Ask one question at a time. Confirm understanding with teach-back.
- Never promise outcomes. Never diagnose or change treatment outside scope.

9.2 Opening Script

'Hello, this is [name], a nurse with [practice]. I am calling for [patient name] as part of your care management program. Can you confirm your date of birth for me? Is this a good time to talk for about fifteen minutes?'

If the patient cannot talk, offer a better time and note it. If another person answers, do not disclose any health information unless the patient has authorized that person.

9.3 Closing Script

'To summarize: you will [action 1] and [action 2]. I will [action 3] by [date]. If you notice [warning signs], call us right away at [number]. Outside office hours, call [after-hours number]. For a medical emergency, call emergency services. When is a good time for me to call next?'

9.4 Channels and Rules

Channel	Allowed use	Rules
Phone	Primary channel	Verify identity first; no detailed health information in voicemail
Secure portal message	Plans, summaries, reminders	Response within 1 business day
Text message	Appointment reminders and non-sensitive nudges	Only with documented consent; no diagnoses or results
Video	Complex education, visual checks	Use the practice's approved platform
Mail	Care plan copies, letters	For patients without digital access
Caregiver contact	As authorized	Authorization recorded in chart

9.5 Handling Difficult Conversations

- Patient refuses medicines: explore reasons without judgment, record the reason, notify the provider.
- Patient is upset about cost: acknowledge, explain the cost-sharing information given at enrollment, offer to connect them with the billing lead, and remind them they may stop at the end of the month.
- Caregiver is overwhelmed: validate, identify one concrete support, and flag for social services referral.
- Patient mentions abuse or neglect: follow the practice's mandatory reporting policy and notify the provider immediately.

10. Medication Management

10.1 Goals

Every patient has one accurate, current medication list that the patient, the team and other providers all share. No change in medicines leaves the practice without a documented reason.

10.2 Medication Reconciliation Procedure

1. Gather sources: EHR list, pharmacy fill history, hospital discharge medication list, specialist notes and the patient's own bottles.
2. Compare every item across sources. Mark each as continue, changed, new, stopped or unclear.
3. For each item record: name, dose, route, frequency, purpose, prescriber, start date and refill status.
4. Identify discrepancies: duplicates, missing drugs, wrong doses, drugs that were supposed to stop.
5. Escalate discrepancies to the provider the same day when they carry safety risk, otherwise within 2 business days.
6. Update the EHR list once the provider confirms. Send the updated list to the patient and pharmacy.
7. Record the date and name of the person who reconciled.

10.3 When Reconciliation Is Mandatory

- At enrollment
- After every hospital, emergency department or skilled nursing stay (within 2 business days of discharge contact, and no later than 7 days)
- When any new prescriber adds or stops a medicine
- At least every 3 months for yellow and red tier patients
- At least every 6 months for green tier patients

10.4 Adherence Support Checklist

- [ ] Patient can read labels and open containers
- [ ] Weekly or daily pill organizer in use if appropriate
- [ ] Refill dates aligned at one pharmacy where possible

- [ ] Reminders (alarms, phone, caregiver) set up
- [ ] Cost barriers screened and referred to assistance programs
- [ ] Side effects discussed and reported to the provider
- [ ] Patient knows what each drug is for
- [ ] High-risk medicines identified (anticoagulants, insulin, opioids, multiple sedatives) with extra monitoring

10.5 Scope Rule

Coordinators identify, document and escalate. They do not change doses, start or stop prescriptions unless a provider order or approved standing protocol allows it, and every such action must be documented with the order reference.

11. Care Coordination and Transitions of Care

11.1 Coordination Principles

The coordinator is the hub. Information about the patient must flow in two directions: from the practice to every other treating clinician and service, and from them back into the chart.

11.2 Referral and Specialist Workflow

1. The provider orders the referral. The coordinator confirms it was sent with the clinical question and relevant records.
2. The assistant or coordinator tracks scheduling. If no appointment exists within 7 days, call the specialist office.
3. Confirm transport and any prep (fasting, paperwork) with the patient 2 days before.
4. After the visit, request the consult note within 14 days. Do not close the referral until the note is received and reviewed by the provider.
5. Update the care plan and medication list with any changes. Tell the patient what changed and why.

11.3 Hospital, Emergency and Facility Transitions

Transitions are the highest-risk moments in a chronic patient's journey. Follow this standard exactly.

Timeframe	Action	Owner
Same business day as notification	Confirm admission or visit; obtain facility contact; flag patient red	Coordinator
During stay	Contact facility case manager; share medication list and care plan; ask expected discharge date	Coordinator
Within 2 business days of discharge	Contact patient or caregiver; confirm they are home, safe and have medicines; screen symptoms	Coordinator
Within 2 business days of discharge	Obtain and review discharge summary; reconcile medicines	Coordinator and provider
Within 7 to 14 days of discharge	Follow-up visit with provider booked and attended (timing depends on risk and applicable transitional care rules)	Front desk and coordinator
Within 7 days	Care plan updated; red-tier status reviewed	Coordinator and provider
30 days	Confirm no readmission; adjust tier	Coordinator

Note: if the practice bills a transitional care service for the same discharge, the billing lead must verify which services may be billed together and in what order under current payer rules.

11.4 Community and Social Resource Workflow

1. Screen needs during the assessment and at each monthly contact.
2. Maintain a current resource directory (food, transport, housing, utilities, caregiver support, mental health, legal aid, senior services).
3. Make a warm referral: call or send details, not just give a phone number, whenever possible.
4. Follow up within 14 days to confirm the patient connected with the service.
5. Record the outcome in the care plan.

11.5 Information Sharing Rules

- Share only the minimum information required for treatment and coordination.
- Use secure, approved channels only.
- Log every external communication: date, who, topic, outcome.

12. Risk, Escalation and Emergency Protocols

12.1 Escalation Levels

Level	Meaning	Examples	Required response time
Emergency (Level 1)	Possible immediate danger to life	Chest pain, severe breathlessness, stroke signs, unresponsiveness, severe allergic reaction, suicidal intent with plan	Direct patient to emergency services immediately; notify provider as soon as the call ends
Urgent (Level 2)	Serious change needing clinical decision within the day	Readings beyond urgent thresholds, fall with injury, new confusion, medication error	Provider contacted within 1 hour; same-day plan
Soon (Level 3)	Concerning trend needing review within days	Rising weight, repeated missed medicines, new side effect, caregiver crisis	Provider review within 2 business days
Routine (Level 4)	No safety concern	Refill request, scheduling	Handle within standard turnaround

12.2 Emergency Call Procedure

1. Stay calm and stay on the line. Ask: 'Are you safe right now? Where are you?'
2. Confirm the patient's location and phone number.
3. Tell the patient to call emergency services, or call them from a second line while keeping the patient on the phone.
4. Do not give clinical instructions beyond emergency protocols approved by the medical director.

5. Notify the billing practitioner or on-call provider immediately.
6. Document the event the same day: time, what was said, actions taken and outcome.
7. Follow up with the patient and facility within 24 hours.

12.3 24/7 Access Standard

- A live person or answering service must be reachable by every enrolled patient at any hour.
- The on-call provider must have electronic access to the patient's care plan and medication list.
- The practice tests the after-hours line monthly and records the result.

12.4 Mental Health Safety Protocol

1. Any statement about wanting to die or harm oneself is treated as an emergency until a provider decides otherwise.
2. Ask directly and calmly about thoughts, plan and means. Do not leave the patient alone on the line if there is imminent risk.
3. If there is immediate danger, contact emergency services and the on-call provider.
4. Provide crisis line information appropriate to the patient's country.
5. Document verbatim statements and all actions. Schedule a follow-up contact within 24 hours.

12.5 Incident Reporting

Any medication error, missed escalation, privacy breach, patient injury or near miss is reported in the incident system within 24 hours. The program manager reviews every incident within 5 business days and reports trends at the monthly quality meeting.

13. Time Tracking and Documentation

13.1 Why This Matters

CCM is billed on documented time and documented required elements. If it is not in the chart, it did not happen for audit purposes. Documentation is part of the work, not an afterthought.

13.2 What Counts as Care Management Time

Countable time is clinical staff or provider time spent on care management for that patient in that calendar month, directed by the care plan. Typical countable activities:

- Patient and caregiver phone, video or secure-message contact about the care plan
- Chart review specific to the patient's chronic conditions
- Medication reconciliation and adherence support
- Care plan creation, revision and review
- Coordination with other providers, pharmacies and community services
- Patient education and self-management support
- Managing care transitions

Time that generally does not count:

- Purely clerical work, such as scheduling or faxing, done by non-clinical staff
- Time spent on the initial enrollment explanation in a way the payer excludes
- Face-to-face visit time that is billed under another service
- Time double-counted by two staff members working at the same moment

The billing lead publishes the current countable-activity list each January and posts it in the team workspace.

13.3 Time Tracking Rules

1. Log time in real time with start and stop, not estimated at the end of the day.
2. Record the staff member's name and credentials with each entry.
3. Count minutes per patient per calendar month; do not carry minutes across months.
4. When two staff members work with the patient together, count the time once, not twice (unless current rules specify otherwise).
5. Round only as the billing lead's policy allows; never inflate.
6. Staff must not log time for tasks they did not personally perform.

13.4 Required Content of Each Encounter Note

Element	Example
Date and time	12 March, 10:15 to 10:34

Element	Example
Staff name and credential	J. Smith, RN
Contact method and person spoken to	Phone, patient
Reason and activities	Monthly review: BP log, medicine check, transport arranged
Findings	Home BP readings within goal; missed 2 doses of evening medicine
Actions taken	Pill organizer reviewed; pharmacy refill sync requested
Escalations	None, or provider notified at 11:00
Plan and next contact date	Next call 26 March
Total minutes	19

13.5 Documentation Quality Rules

- Notes are completed within 24 hours of the encounter.
- Use objective statements. Avoid vague phrases like 'patient doing fine'.
- Never copy and paste a prior note without editing; cloned notes are an audit risk.
- Corrections are made by addendum with date and reason; never delete the original.
- Every entry is signed electronically.

14. Billing and Compliance (Medicare Framework)

14.1 Service Types and Time Thresholds

The following reflects the commonly used Medicare CCM code families. Codes, time thresholds and payment rates are updated regularly, so the billing lead must verify current definitions before each calendar year and whenever the payer issues changes.

Service	Typical code	Core requirement	Who delivers
Non-complex CCM, first 20 minutes	99490	At least 20 minutes of clinical staff time in the month	Clinical staff under general supervision
Non-complex CCM, each additional 20 minutes	99439	Add-on; each further 20 minutes, with a payer limit on units per month	Clinical staff
Complex CCM, first 60 minutes	99487	At least 60 minutes of clinical staff time and moderate or high complexity medical decision making	Clinical staff with provider direction
Complex CCM, each additional 30 minutes	99489	Add-on to complex CCM	Clinical staff
CCM personally performed by a physician or qualified provider, first 30 minutes	99491	At least 30 minutes of the billing practitioner's own time	Physician or qualified provider
Add-on for personally performed CCM, each additional 30 minutes	99437	Add-on to the personally performed service	Physician or qualified provider

Only one CCM family may be billed for a patient in a month in the combinations the payer permits. The billing lead resolves any overlap before a claim is submitted.

14.2 Pre-Claim Compliance Checklist

Every claim must pass each item. If any item fails, the claim is held.

- [ ] Patient had an initiating visit within the required period
- [ ] Consent documented, including the five required disclosures
- [ ] Two or more qualifying chronic conditions documented by the provider
- [ ] Comprehensive care plan exists, is signed and was given to the patient
- [ ] Time thresholds met by qualifying activities in the same calendar month
- [ ] Time logged by appropriately qualified staff under supervision

- [ ] 24/7 access communicated and functional
- [ ] No other practitioner billing CCM for the patient this month
- [ ] No conflicting service the payer disallows in the same period
- [ ] Encounter notes complete, signed and consistent with logged time
- [ ] Diagnosis codes reflect documented chronic conditions
- [ ] Date of service and rendering and billing provider recorded correctly

14.3 Claim Submission Procedure

1. On the first business days of the following month, the billing lead exports a time summary per patient.
2. The lead cross-checks each summary against encounter notes and the checklist in 14.2.
3. Qualifying claims are entered, with the date of service following payer rules.
4. Claims that fail are returned to the coordinator with a specific fix request and a due date.
5. Submitted claims are tracked for payment, denial and patient cost share.
6. Denials are reviewed within 5 business days, corrected and resubmitted if appropriate.

14.4 Patient Billing and Cost Sharing

- Patients are informed of the expected cost during enrollment.
- Every statement and call about billing is handled by the billing lead or trained front-desk staff.
- Do not waive cost sharing as a routine practice without confirming it is permitted; regulators generally treat blanket waivers as a compliance risk. Seek compliance advice before any hardship policy.
- Offer payment plans or assistance information when a patient raises cost as a barrier.

14.5 Compliance Safeguards

- Quarterly internal audit of at least 10 percent of billed patients, sampled across coordinators.
- Zero tolerance for logging time that was not worked.
- Annual compliance training for every team member.
- Any billing error found is corrected and repaid promptly according to policy and legal advice.

15. Quality Measurement and Continuous Improvement

15.1 Program Scorecard

The program manager produces this scorecard monthly and shares it at the quality meeting.

Domain	Measure	Target
Reach	Percent of eligible patients enrolled	Set by practice, review quarterly
Engagement	Percent of enrolled patients contacted in the month	95 percent or higher
Timeliness	Percent of post-discharge contacts within 2 business days	90 percent or higher
Planning	Percent of patients with a signed care plan within 30 days of enrollment	95 percent or higher
Clinical control	Percent meeting condition-specific targets (blood pressure, HbA1c and others)	Improve versus baseline each quarter
Utilization	Emergency visits and admissions per 1,000 enrolled patients	Decrease versus baseline
Medication safety	Percent with reconciled list in the last 90 days	95 percent or higher
Experience	Patient satisfaction score and complaint count	Target set by practice
Compliance	Percent of audited claims with no documentation defects	98 percent or higher
Retention	Disenrollment rate and reasons	Track and review

15.2 Improvement Cycle

1. Review the scorecard every month.
2. Select no more than two measures that miss target.
3. Examine root causes with the team: staffing, scripts, tools, patient mix.

4. Test one change for 30 to 60 days.
5. Measure whether the number moved. Keep what works; drop what does not.
6. Record the change in the program log.

15.3 Case Review Meeting

Held weekly, 30 minutes. Review: every red-tier patient, every unplanned hospital visit, every escalation and every complaint. Ask for each: what happened, was the workflow followed, what could have prevented it, what changes now.

16. Privacy, Security and Patient Rights

16.1 Core Rules

- Access patient records only when needed for your assigned work.
- Verify identity with two identifiers before sharing information.
- Use only approved, encrypted tools for messages and file transfer; no personal messaging apps for patient information.
- Lock screens when stepping away. Never share logins.
- Do not discuss patients in public areas.
- Report suspected breaches to the privacy officer immediately, not later.

16.2 Authorized Representatives

Record in the chart each person the patient allows the team to speak to, what information they may receive and the date of authorization. Review authorizations annually and whenever the patient asks.

16.3 Patient Rights

Patients may at any time: stop CCM (effective at the end of the calendar month), obtain a copy of the care plan, request corrections to their record, choose how they are contacted, and make a complaint without penalty to their care.

16.4 Complaint Handling

1. Acknowledge the complaint within 1 business day.
2. Log it with date, summary and owner.

3. Investigate and respond in writing within 14 days.
4. Record the outcome and any process change.

17. Disenrollment and Program Exit

17.1 Reasons for Exit

- Patient request
- Patient no longer meets eligibility
- Patient moves or transfers to another practice
- Patient enters a service that cannot overlap with CCM (for example hospice)
- Loss of coverage
- Patient unreachable for 90 days despite documented attempts
- Death

17.2 Procedure

1. Record the reason, date and who made the decision.
2. For patient-initiated stops, confirm the end date is the last day of the calendar month and that billing for that month remains accurate.
3. The provider reviews clinical risk before exit and decides if a bridging plan is needed.
4. Send a closing letter and a final copy of the care plan to the patient and, with permission, to the new provider or practice.
5. Hand over any open referrals or tasks to the primary care team.
6. Update the registry and stop outreach tasks.
7. If the patient is transferring to another practice that will start CCM, confirm the transfer month so that only one practitioner bills in any one month.

17.3 Re-Enrollment

A patient who left may re-enroll. Repeat the eligibility screen and consent steps. Reuse the previous assessment as a starting point but update it within 14 days.

18. Training and Onboarding of Team Members

18.1 Core Training Plan

Week	Content	Sign-off
1	Program overview, privacy, this guide Sections 1 to 5, EHR navigation	Program manager
2	Assessments, care plans, medication reconciliation (Sections 6, 7, 10) with shadowing	Senior coordinator
3	Monthly cycle, communication scripts, escalation (Sections 8, 9, 12) with supervised calls	Senior coordinator
4	Transitions, documentation and time tracking, billing basics (Sections 11, 13, 14)	Billing lead
5 to 6	Independent calls with 100 percent note review	Program manager
8	Competency assessment and caseload assignment	Medical director

18.2 Ongoing Requirements

- Annual refresher on privacy, compliance and emergency protocols
- Quarterly skills session (motivational interviewing, teach-back, condition updates)
- Immediate retraining when rules change or audit finds gaps
- Documented attendance for every session

18.3 Competency Checklist for New Coordinators

- [ ] Completes an enrollment conversation with all five disclosures
- [ ] Completes an assessment with a correct risk tier
- [ ] Reconciles a medication list without errors on a test case
- [ ] Writes a care plan with measurable goals
- [ ] Demonstrates correct escalation on three simulated scenarios
- [ ] Logs time and writes notes meeting documentation standards
- [ ] Completes a post-discharge call within the standard
- [ ] Passes privacy and compliance quiz

Appendix A. Quick-Reference Workflow Summary

Stage	Trigger	Action	Time standard	Owner
Identify	Monthly registry run or referral	Screen clinical and administrative eligibility	Within 5 days of identification	Coordinator and billing lead
Enroll	Eligible candidate	Explain program, record consent	Same conversation	Coordinator
Welcome	Consent given	Send packet and give 24/7 number	5 business days	Care team assistant
Assess	Enrollment	Full assessment and risk tier	14 days	Coordinator
Plan	Assessment complete	Draft, sign, share care plan	5 business days to draft; provider signs promptly	Coordinator and provider
Manage	Every month	Monthly cycle (Section 8)	Per tier	Coordinator
Transition	Admission or ED visit	Follow transitions table	2 business days post-discharge	Coordinator
Escalate	Any red flag	Section 12 protocol	Per level	Anyone on the team
Bill	Month end	Compliance checklist and claim	First 3 business days of next month	Billing lead
Review	Monthly and quarterly	Scorecard and audits	Per calendar	Program manager
Exit	Disenrollment trigger	Section 17	End of the calendar month	Coordinator and provider

Appendix B. Template: Initial Assessment Form (Headings)

1. Patient identifiers, contact preferences, language, interpreter need
2. Insurance and eligibility confirmation

3. Consent record (date, method, staff, disclosures confirmed)
4. Chronic conditions with status and last control marker
5. Symptom review by condition
6. Reconciled medication list
7. Allergies and intolerances
8. Functional status and falls risk
9. Cognition and mood screens
10. Social needs and caregiver details
11. Preventive care status
12. Advance care planning documents
13. Patient goals (verbatim)
14. Risk tier and rationale
15. Care team directory
16. Provider review signature and date

Appendix C. Template: Monthly Encounter Note

- Patient name, date of birth, record number
- Date, start and stop time, total minutes
- Staff name and credential
- Contact method and person spoken to
- Status since last contact
- Condition checks and readings
- Medication review and changes
- Appointments, referrals and tests
- Social and caregiver needs
- Education given and teach-back result
- Escalations and provider notified (name and time)
- Care plan changes made
- Tasks created with owner and due date
- Next contact date

Appendix D. Template: Post-Discharge Contact Call

1. Confirm identity and that the patient is home.
2. Ask how they are feeling and whether any symptom is worse than before the stay.
3. Confirm they have all discharge medicines and understand each one.
4. Compare the discharge list with the previous list and note every change.
5. Confirm follow-up appointments, transport and who is helping at home.
6. Review warning signs and who to call.
7. Escalate unresolved issues to the provider the same day.
8. Record time, outcome and next contact date.

Appendix E. Monthly Billing Readiness Checklist

- [ ] Time log exported and reconciled with notes
- [ ] Every patient meets the minimum threshold for the chosen service
- [ ] Consent and care plan present for every billed patient
- [ ] Initiating visit confirmed
- [ ] Overlapping services reviewed
- [ ] Provider sign-off on complex or physician-performed services
- [ ] Claims entered and submitted
- [ ] Denials from prior month reviewed
- [ ] Audit sample selected

Appendix F. Document Control

Item	Detail
Document owner	Program Medical Director
Approver	Practice leadership
Effective date	To be entered on approval
Next review	12 months after effective date, or sooner on any rule change

Item	Detail
Change log	Record every revision with date, author and summary

This guide is an operational template. Before use, the practice should adapt names, thresholds, escalation numbers and billing details to its own policies, local law and the current rules of its payers, and have clinical and compliance leaders approve it.